

1. Administrative & Study Identification

**Full Study Title:** Retrospective Study for Describing Real-World Effectiveness of Nivolumab + Chemotherapy in Neoadjuvant NSCLC in Argentina **Short Title/Acronym:** CA209-1524 **Protocol Number:** CA209-1524 **NCT Number:** NCT07141563 **Posting ID:** 266013830282742088 **Trial Status:** ENROLLING\_BY\_INVITATION **Sponsor Name:** Bristol Myers Squibb **Company:** Bristol Myers Squibb **Investigational Product:** Nivolumab (Opdivo) + platinum-based chemotherapy **Phase of Development:** Observational / Retrospective (Phase Not Applicable) **Medical Monitor:** Bristol Myers Squibb Clinical Operations Lead

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2. Study Synopsis & Rationale

**2.1 Study Objective Primary Objective:** To estimate the rate of pathological complete response (pCR) following neoadjuvant therapy. **Secondary Objectives:** To evaluate major pathological response (MPR), event-free survival (EFS) over 1 year, surgical outcomes, safety, participant demographics, and subsequent adjuvant therapy use.

**2.2 Background & Rationale** To describe the real-world effectiveness and safety of neoadjuvant nivolumab plus platinum-based chemotherapy in Argentinian patients with resectable non-small cell lung cancer (NSCLC). This research is conducted to understand if this treatment combination serves as a reliable option in real-life settings outside of controlled clinical trials, utilizing historical data from medical records.

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3. Study Design & Methodology

**3.1 Design Framework Study Type:** Observational (Non-interventional, retrospective, single-arm, multicenter study) **Control Type:** Single-arm **Blinding:** Open-label (Retrospective chart review) **Randomization:** N/A

**3.2 Planned Enrollment & Duration Target \$N\$:** 80 patients **Number of Sites:** Multicenter (Participating Argentinian centers) **Estimated Study Start:** 26-May-2025 **Estimated Primary Completion:** 18-Nov-2025

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4. Subject Selection (Eligibility Criteria)

**4.1 Inclusion Criteria** 1. Participants aged 18 years or older. 2. Participants diagnosed with resectable non-small cell lung cancer

(NSCLC) (stage IIA-IIIB TNM AJCC 8th ed) according to the treating team's judgment. 3. Participants who received nivolumab (360 mg Q3W) and platinum-based chemotherapy as neoadjuvant therapy between July 1, 2022, and March 1, 2025. 4. Participants must have given informed consent for their data to be used for research or academic studies, or a waiver of consent has been obtained.

**4.2 Exclusion Criteria** 1. Participants with confirmed positive EGFR gene mutation or ALK fusion gene. 2. Prior malignancy active within the previous 3 years, except for locally curable cancers that have been apparently cured (e.g., basal or squamous cell skin cancer, superficial bladder cancer, or carcinoma in situ of the prostate, cervix, or breast). 3. Participants who received any antineoplastic therapy within 3 months prior to initiating neoadjuvant nivolumab and chemotherapy. 4. Participants who participated in a clinical trial for NSCLC. 5. Participants with complications of treatment-related adverse events related to hematotoxicity of antineoplastic agents 3 months prior to the start of neoadjuvant therapy. 6. Participants who started neoadjuvant therapy outside the participating medical institution and were transferred to the institution for surgery.

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## 5. Intervention & Treatment Plan

**5.1 Investigational Regimen Dosage/Frequency:** Nivolumab 360 mg Q3W (every 3 weeks) plus platinum-based chemotherapy. **Route of Administration:** Intravenous (standard for administration of nivolumab and systemic chemotherapy). **Duration of Treatment:** Neoadjuvant phase prior to surgery; patient data is extracted for up to 12 months following surgery or neoadjuvant therapy.

**5.2 Concomitant & Prohibited Therapy Permitted:** Standard supportive care and subsequent adjuvant therapies per real-world practices. **Prohibited:** Antineoplastic therapy within 3 months prior to initiating neoadjuvant nivolumab + chemotherapy.

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## 6. Study Timeline & Visit Schedule

*(Note: As a retrospective study, "visits" represent data extraction points from historical medical records.)*

|                                                                      |                |                                                                                |                                                                                                                          |
|----------------------------------------------------------------------|----------------|--------------------------------------------------------------------------------|--------------------------------------------------------------------------------------------------------------------------|
| Visit ID                                                             | Window (Days)  | Key Activities/Procedures                                                      | :---                                                                                                                     |
| :---                                                                 | :---           | <b>Screening</b>                                                               | N/A   Identification of eligible patients treated between Jul 2022 and Mar 2025     <b>Baseline</b>   N/A                |
| Extraction of demographic, diagnosis, and pretreatment clinical data | <b>Interim</b> | N/A   Extraction of neoadjuvant treatment administration and surgical outcomes | <b>End of Study</b>   Up to 12 months post-surgery   Extraction of 1-year EFS, recurrence, and safety/adverse event data |

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## 7. Outcome Measures (Endpoints)

**7.1 Primary Endpoint Definition:** Rate of pathological complete response (pCR) post-surgery, defined as the number of patients with complete pathological response (absence of residual viable tumor in both lung and lymph nodes at the time of surgery) divided by the total number of patients. **Measurement Method:** Retrospective review of post-surgical pathology reports.

**7.2 Secondary & Exploratory Endpoints** \* Event-free survival (EFS)  
\* Rate of major pathological response (MPR) post-surgery \* Participant demographics \* Surgical outcomes, real-world safety profiles, and subsequent adjuvant therapy use

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## 8. Safety & Adverse Event Reporting

**Adverse Event (AE) Monitoring:** Retrospective data extraction from medical records for safety and treatment-related adverse events documented by the treating physician. **Serious Adverse Events (SAEs):** Evaluated based on historical medical charts during the observation period. **Data Monitoring Committee (DMC):** N/A for retrospective observational chart reviews.

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## 9. Statistical Analysis Plan (SAP) Summary

**Analysis Populations:** Real-world patient cohort meeting all inclusion/exclusion criteria. **Sample Size Justification:** Descriptive sample of 80 evaluable records based on available data from participating Argentinian centers. **Handling of Missing Data:** Standard handling for retrospective descriptive and survival analyses; missing data will be noted as not reported in electronic medical records.

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## 10. Quality Assurance & Regulatory Compliance

**GCP Compliance:** Conducted in accordance with Good Pharmacoepidemiology Practices (GPP) and local ethical regulations regarding the secondary use of medical records. **Monitoring Plan:** Source data verification through electronic health record curation. **Audit Trail:** Secured and anonymized database for retrospective cohort extraction.

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## 11. Study Identifiers & Governance

**Primary ID:** CA209-1524 **Registry Number:** NCT07141563 **Sponsor/Lead Organization:** Bristol Myers Squibb **Study Title:** A Study Describing Real-World Effectiveness of Nivolumab + Chemotherapy in Neoadjuvant NSCLC **Official Regulatory Title:** Retrospective

12. Clinical Framework (NSCLC Specific)

**Disease Area:** Non-small Cell Lung Cancer (NSCLC) **Primary Condition:** Non-small Cell Lung Cancer (NSCLC) **Preferred UMLS Name:** Nivolumab for Non-small Cell Lung Carcinoma **Diagnosis Threshold:** Resectable, early-stage (Stage IIA-IIIB, AJCC 8th ed) without EGFR/ALK driver mutations **Medical Methodology:** Neoadjuvant Immunotherapy + Chemotherapy **Optimization Target:** Pathological Complete Response (pCR) prior to surgical resection

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13. Study Procedures & Methodology

**Management Pathway:** Retrospective chart review of the standard clinical pathway for neoadjuvant NSCLC treatment **Education Protocol (HCP):** N/A (Retrospective observation) **Education Protocol (Patient):** N/A (Retrospective observation) **Quality Audit Mechanism:** Medical record data validation and curation

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14. Observation & Follow-Up Schedule

**Total Duration:** Up to 12 months following surgery or neoadjuvant therapy **Onsite Visit Cadence:** N/A (Retrospective Data Collection) **Remote Monitoring Frequency:** N/A **Checklist Review Interval:** N/A

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15. Sign-off & Approval

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|--------------------------|--------|-----------|---------------|------|------|------|------|
| Role                     | Name   | Signature | Date          | :--- | :--- | :--- | :--- |
| Principal Investigator   | [Name] | _____     | [DD-MMM-YYYY] |      |      |      |      |
| Clinical Operations Lead | [Name] | _____     | [DD-MMM-YYYY] |      |      | Lead |      |
| Statistician             | [Name] | _____     | [DD-MMM-YYYY] |      |      |      |      |